PROCEDURAL SEDATION FAQS FOR NON– ANESTHESIOLOGISTS

The regulatory requirement around deep sedation falls under the Anesthesia section CMS §482.52 which includes requirements for pre, intra and post Anesthesia evaluations. The more stringent requirements at §482.52(b)(1) and §482.52(b)(3) do not permit delegation of the pre-and post-anesthesia evaluation to practitioners who are not qualified to administer anesthesia.

1

Why does the MD need to push the medication? The RN is able to push the same medication during Rapid Sequence Induction (RSI), but not sedation. This doesn't make sense.

CMS only allows physicians with deep sedation privileges to administer medications and monitor the patient when undergoing deep procedural sedation. RNs are not privileged for deep sedation and cannot administer/push medications or monitor the patient having deep sedation for a procedure.

Note: RSI is considered to be an Emergent Situation and nurse administration may be necessary under the direct supervision of the treating physician.

2

Do both MDs need to be in the room until the patient recovers? Or once the procedure is done can one of the MDs leave the room?

Two providers need to be involved in the care of the deep sedation patient. One physician to provide the deep sedation and the second physician/provider to perform the actual procedure. The physician performing the procedure can leave as soon as the procedure is complete, however, the physician providing deep sedation must remain to monitor the patient until the patient emerges from the level of deep sedation. Once the patient is no longer deeply sedated, the RN with sedation competencies can recover the patient.

3

Do we know what the other EDs are planning on doing? Are they calling anesthesia down?

It's up to each medical center to figure out their local workflows. No medical center has the resources to have anesthesia available on standby for cases everywhere deep sedation is currently being administered. Facilities that do not currently have the staffing or ability to call in two providers in order to provide compliant procedures under deep sedation will have to consider all possibilities. One possible solution is to place the patient in the OR queue after NPO requirements are fulfilled. However, consider that this will impact OR access and potentially delay patient care. Again, this should be addressed at the medical center level involving anesthesia, ED and administrative leadership.

Note: In an Emergent Situation such as cardioversion of unstable arrhythmias or reduction of an extremity with neuro vascular compromise the physician must do what is medically necessary at the time.

4

Since the new policy has not been updated and the current policy does not state two MDs need to be in the room, do I have time to work out our work flow before we start this new process?

The current policy is clear about two points: (1) an RN cannot administer deep sedation and (2) the provider administering sedation/monitoring the patient can have no other duties that would cause the patient to be unattended or compromise continuous monitoring.

To clarify, Two MDs do not need to be in the room the entire time the patient is having deep sedation. One physician with deep sedation privileges needs to administer medication and continuously monitor the patient. Another provider privileged to perform the intended procedure needs to perform the procedure. Once the procedure is completed, the provider who performed the procedure does not need to stay in attendance with the patient. Once the patient has emerged from the level of deep sedation, the physician that administered the deep sedation can hand-off to an RN with sedation competencies who can recover the patient.

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5

Can we redefine Etomidate, Propofol and Ketamine as moderate sedation drugs?
This has been considered. However, Etomidate and Propofol have increased risk of respiratory depression/obstruction. At this time, we are actively exploring redefining ketamine as a dissociative agent (as opposed to deep sedating agent) as it has a much safer respiratory profile.

6

If the Orthopedic Surgeon is performing the procedure (reduction, etc), does the ER physicianstill obtain a signed consent for both the sedation and the procedure (one signed consent)? Then the Orthopedic Surgeon just documents an informed consent in their note, without obtaining a signed consent themselves?
It is the physicians' responsibility to obtain informed consent. The physician who performs the procedure is responsible for obtaining the patient's consent. Both providers that are involved in the procedure/sedation would obtain informed consent - discuss with the patient the indications for the procedure, risks, benefits, and alternatives and get agreement from the patient to proceed. Both providers should be speaking with the patient and documenting Risks/Benefits/Alternatives for their portion of the procedure or sedation being administered.
For example:. The ER physician would document the informed consent for procedural sedation and the orthopedic surgeon or AHP (e.g. orthopedic PA) that is privileged to perform the procedure would document informed consent for the procedure. Only one consent form is required to be signed by the patient that states the nature of the procedure performed under procedural sedation.
The ordering physician shall place an order for the nurse to obtain the patient's signature on the "Consent to Operation, Administration of Anesthetics and the Rendering of Other Medical Services" form or e-consent form. (The consent form is NOT informed consent. It is a document that validates informed consent was obtained. The consent form is to be completed AFTER the informed consent discussion has taken place.)
There is no need to have the patient sign two separate consent forms.
Note: Please refer to hospital policy for details to your hospital.

7

Which organization mentions a two physician requirement for a procedural deep sedation? I just want to know so when someone asks, we can refer them to the appropriate document.
The below position statements from American Society of Anesthesiologists (ASA) state that the privileged provider administer the deep sedation, monitor the patient and be completely dedicated to that task.
STATEMENT ON GRANTING PRIVILEGES TO NONANESTHESIOLOGIST PHYSICIANS FOR PERSONALLY ADMINISTERING OR SUPERVISING DEEP SEDATION (Approved by the ASA House of Delegates on October 18, 2006, and amended on October 17, 2012)
Because of the significant risk that patients who receive deep sedation may enter a state of general anesthesia, privileges for deep sedation should be granted only to nonanesthesiologist physicians who are qualified and trained in the medical practice of deep sedation and the recognition of and rescue from general anesthesia.
Nonanesthesiologist physicians may neither delegate nor supervise the administration or monitoring of deep sedation by individuals who are not themselves qualified and trained to administer deep sedation, and the recognition of and rescue from general anesthesia.
The Statement on Safe Use of Propofol, dated October 15, 2014, that reads,
"The practitioner administering propofol for sedation/anesthesia should, at a minimum, have the education and training to identify and manage the airway and cardiovascular changes which occur in a patient who enters a state of general anesthesia, as well as the ability to assist in the management of complications. The practitioner monitoring the patient should be present throughout the procedure and be completely dedicated to that task".